Fax cover sheet

Bayfront Regional Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home – Recuperative Care Intake
Fax: 555-0199
From: Teresa Lindqvist, RN CM
Phone: 555-0161
Date: 10/06/2026 09:15
Pages (including cover): 14
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

58M community-acquired pneumonia, improving. Room air, ambulating independently. Hoping for d/c to recuperative care 10/06 PM. Please call CM with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Bayfront Regional Medical Center (fictional) Dept/Floor: 3 North Medical
SW/CM/RN: Teresa Lindqvist, RN CM Phone/Pager #: 555-0161
Email: tlindqvist@bayfront.example Nursing Station #: 555-0162
Authorized by: Dr. P. Okonkwo Phone #:

Patient Information

Patient Name: Daniel Doe MRN: BR-6120873 Date of Birth (dd/mm/yyy): 04/12/1968
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Community-acquired pneumonia, RLL
Admit Date: 10/02/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/02/2026 Results: CXR · RLL infiltrate, no cavitation

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes*	X No	Active Infection	X Yes*	No
*Explain			*Explain	pneumonia - On IV antibiotics: Yes	
Self Administer All Meds	X Yes	No*	Ambulatory	X Yes	No
Continent of Bowel & Bladder	X Yes	No	Assistive Device	Yes*	X No
Colostomy/Ileostomy	Yes	X No	*Explain		
Foley Catheter	Yes	X No	Hospice	Yes*	X No
Diabetes	Yes	X No	*Explain		
Insulin	Yes	X No	Car	Yes	X No
Diabetes related Oral Meds	Yes	X No	Service Animal	Yes	X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Daniel Doe

DOB: 04/12/1968

Member ID: OCH-55219036

Referring Provider

Facility: Bayfront Regional Medical Center (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism (RLL)

ICD-10: J18.9

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/06/2026

Requested Days: 14

Admit Date: 10/02/2026

Clinical summary: CAP improving, room air, independent ADLs.

Plan Use Only

Status: Pending review

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Patient face sheet

Bayfront Regional Medical Center (fictional) - PATIENT FACE SHEET

Patient: Daniel Doe
MRN: BR-6120873
Account #: 99120873
DOB: 04/12/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/02/2026 13:05 (ED arrival 10/02/2026 08:40)
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Patience Okonkwo, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 10/02/2026

CC

Cough, fever, shortness of breath x 4 days

HPI

58 y/o M, unhoused x 2 yrs (sleeps in a tent near Example Creek trail), presents with productive cough, fevers and dyspnea on exertion x 4 days. In ED T 102.1F, HR 112, SpO2 89% RA improving to 94% on 2L NC. CXR: RLL consolidation.

Prior level of function: independent, no assistive device, walks daily to meal program.

PMH

- Tobacco use (30 pack-years)
- No known COPD (no prior PFTs)

Meds PTA

None

Allergies

NKDA

Social

Tobacco 1/2 ppd. EtOH rarely, last drink >1 month ago. Denies drug use.

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History and Physical - page 2

Exam

T 101.4F HR 108 BP 128/76 RR 22 SpO2 94% 2L NC
Crackles RLL. No accessory muscle use.
Alert, AOx4, calm and cooperative.

Assessment/Plan

1. Community-acquired pneumonia, RLL (CURB-65 = 1) - ceftriaxone 1 g IV q24h + azithromycin 500 mg IV daily; blood and sputum cultures; droplet precautions pending respiratory viral panel
2. Acute hypoxemia - O2 to keep SpO2 >= 92%, wean as tolerated
3. Tobacco use - nicotine patch, cessation counseling
4. Dispo - SW/CM consult (unhoused)

Signed: Patience Okonkwo, MD 10/02/2026 15:10

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/06/2026 07:40

S: Cough much improved, less sputum. No dyspnea walking in hall. Eating well.

O: T 98.4F HR 82 BP 122/74 RR 16 SpO2 95% RA. Few RLL crackles. AOx4.

Afebrile since 10/03 18:00. WBC 14.8 -> 8.9.

A/P:

1. CAP RLL - clinically improved. IV ceftriaxone/azithromycin STOPPED 10/04; transitioned to PO amoxicillin-clavulanate 875 mg BID through 10/09. Azithromycin course completed 10/05. No IV antibiotics needed. PIV removed 10/05.
2. Hypoxemia - resolved. On room air since 10/04, ambulatory SpO2 95% RA. No home O2 needed.
3. Isolation - respiratory viral panel negative 10/02; droplet precautions discontinued 10/02. No isolation.
4. Dispo - medically stable for d/c to recuperative care today. PCP f/u and repeat CXR in 6 wks.

Signed: Patience Okonkwo, MD

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Medication administration record (excerpt)

MEDICATION ADMINISTRATION RECORD - ANTIBIOTICS

Patient: Daniel Doe MRN: BR-6120873

Ceftriaxone 1 g IV q24h | Start 10/02 11:30 | Given 10/02 11:30, 10/03 11:00, 10/04 10:45 | DISCONTINUED 10/04 12:00

Azithromycin 500 mg IV daily | Start 10/02 12:00 | Given 10/02, 10/03, 10/04 | DISCONTINUED 10/04 12:00

Azithromycin 500 mg PO daily | 10/05 09:00 given | Course complete 10/05

Amoxicillin-clavulanate 875 mg PO BID | Start 10/04 21:00 | Given 10/04 21:00, 10/05 09:00, 10/05 21:00, 10/06 09:00 | through 10/09

IV access

Peripheral IV R forearm - removed 10/05 14:00, site clean

Notes

Pt self-administered PO doses with RN observation 10/05 and 10/06; states dose and schedule correctly.

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Respiratory therapy note

RESPIRATORY THERAPY NOTE

Date: 10/05/2026 16:00

O2 weaned to room air 10/04 08:00. Ambulatory oximetry 10/05: rest SpO2 96% RA, after walking 200 ft SpO2 95% RA, no desaturation below 94%.

No nebulizer or inhaler treatments required. Incentive spirometer 1500 mL, pt independent with use.

No home O2 indicated.

Signed: G. Mbatha, RRT

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Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 10/05/2026 11:00

Bed mobility: independent

Sit<>stand: independent

Transfers: independent, toilet and shower chair transfer independent

Gait: 200 ft x 2, independent, no assistive device, steady; 1 standing rest break

Stairs: 12 steps with one rail, independent

Balance: good static and dynamic

Assessment

At baseline. No skilled PT needs. No DME needed.

Recommendation

D/c to recuperative care or community without PT follow-up.

Signed: H. Varga, PT, DPT

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Nursing shift note

NURSING SHIFT NOTE

Date: 10/06/2026 06:30

Pt AOx4. Independent with all ADLs. Showered independently 10/05 PM, gathered own supplies. Continent of bowel and bladder, toilets independently.

Self-administers PO meds, teach-back done on amoxicillin-clavulanate schedule.

Calm and cooperative. No behavioral concerns this admission.

Isolation status: none (standard precautions).

Room air, SpO2 95%. Afebrile.

Signed: C. Duarte, RN

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Social work note

SOCIAL WORK NOTE

Date: 10/05/2026 13:30

Pt engaged and pleasant, AOx4. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history.

No conservatorship. Not enrolled in FSP. Not a registered sex offender per pt.

Unhoused x 2 yrs; tent and belongings at encampment. Agreeable to recuperative care, then housing navigation referral.

Signed: Teresa Lindqvist, RN CM / S. Achterberg, MSW

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Labs, imaging and vitals

LABS

10/02 | WBC 14.8 H | Hgb 13.9 | Plt 301 | Na 136 | K 4.0 | Cr 1.0 | BUN 18 | Procalcitonin 1.2 H
10/04 | WBC 11.2 H
10/06 | WBC 8.9 | Cr 0.9
10/02 Blood cultures x2: no growth at 72 hours (final pending)
10/02 Sputum culture: Strep pneumoniae, ceftriaxone and amoxicillin susceptible
10/02 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/02 MRSA nares PCR: Negative
10/06 05:40 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/02 CXR PA/lat: RLL consolidation, no effusion, no cavitation, no pneumothorax
10/05 CXR portable: improving RLL opacity

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/05 08:00	98.2	84	124/76	16	95%	RA
10/05 14:00	98.6	86	126/78	16	96%	RA
10/05 20:00	98.4	80	120/72	16	95%	RA
10/06 02:00	98.0	76	118/70	14	95%	RA
10/06 07:40	98.4	82	122/74	16	95%	RA

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Daniel Doe MRN: BR-6120873

- Amoxicillin-clavulanate 875 mg PO BID through 10/09/2026 (6 doses remaining)
- Nicotine patch 14 mg/24h daily
- Acetaminophen 650 mg PO q6h PRN fever/pain
- Guaifenesin 600 mg PO BID PRN cough

No IV medications. No home O2.

Patient able to self-administer all medications: Yes

Meds-to-beds: full supply dispensed by inpatient pharmacy at discharge

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